Stock pitch · Cautionary case from the systematic value portfolio

Centene

NYSE: CNC · Health Care (managed care) · United States · Price $67.60 (29 Oct 2026)

NEUTRAL · case study

When the screen is cheap for the wrong reason: Centene looked like the cheapest health-care stock in December 2025 because the model was still reading 2024 earnings.

Noah Doose · 29 October 2026 · Student research for interview purposes, not investment advice. Not affiliated with, endorsed by or prepared for any firm.

Key takeaways

A lesson in process rather than a recommendation: trailing value can be an illusion when earnings are being revised.

1. The model saw a 5.6x P/E. With a six-month accounting lag, the December 2025 score used fiscal 2024 earnings. The 2025 reality was $2.08 after a guidance withdrawal and a $6.7bn goodwill write-down.

2. The outcome was good, the reasoning was not. The stock returned +62% in the first half of 2026 as 2026 guidance rose from > $3.00 to > $5.20 — a recovery the value signal did not forecast.

3. Today it is fairly priced. At about 13x the 2026 earnings floor, and with ~$0.50 of one-offs inside it, we would neither add nor chase. Our call: neutral, hold at the model weight.

Source: Centene 8-Ks (1 Jul 2025, 6 Feb 2026, 28 Jul 2026, 27 Oct 2026), SEC EDGAR; pipeline score at 31/12/2025 (fiscal year used: 2024-12-31)

Why the model selected it

Metric (z-score within Health Care)	Centene	Managed-care peers (median multiple)
EV/EBIT yield	+3.19	EV/EBIT 9.8x vs Centene 6.8x
Book-to-price	+4.06	P/B 1.66x vs 0.68x
FCF yield	-1.77	negative FCF in the fiscal year used
Sales-to-EV	+4.38	EV/Sales 0.39x vs 0.13x
Composite score / rank	2.47	rank 1 of 108

Two structural biases: (i) insurers have thin margins and huge revenues, so EV/Sales flags every health insurer as cheap against medtech or pharma in the same GICS sector; (ii) the lag means the score reacts to bad news 12-18 months late.

Peers: Elevance, Humana, Cigna, UnitedHealth, CVS.

Source: pipeline z-scores and multiples at 31/12/2025 (last usable fiscal year, Bloomberg-derived)

Business and economic drivers

What it does: the largest U.S. Medicaid managed-care company, plus Medicare (Wellcare) and the ACA Marketplace (Ambetter). Revenues $195bn in 2025; total membership 25.7m (Sep 2026).

How it earns money: fixed premiums from states and the federal government minus medical costs: a 1-point move in the health benefits ratio (92.7% in Q3 2026) is worth about $1.7bn of pre-tax profit.

What went wrong in 2025: a sicker Marketplace risk pool and lower risk-adjustment transfers (~$1.8bn), and rising Medicaid acuity (behavioural health, home health, specialty drugs).

Policy as the main driver: expiry of enhanced ACA subsidies (Marketplace members 5.8m → 3.4m), Medicaid work requirements from 2027 (One Big Beautiful Bill Act), annual Medicare rate notices.

Source: Centene 10-K FY2025 and 10-Q Q3 2026 (SEC EDGAR); 8-K 1 Jul 2025; own sensitivity on premium revenue

How guidance moved

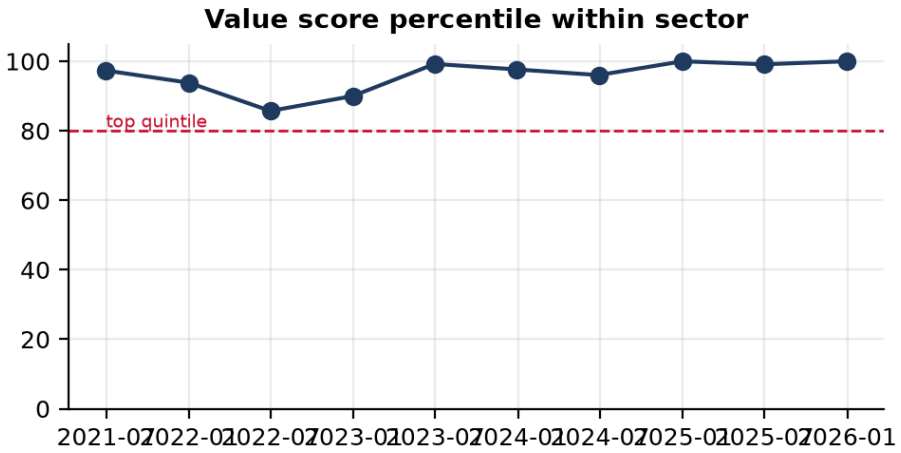
Date	Event	Adjusted EPS guidance
Dec 2024	Initial 2025 guide	> $7.25 (to verify)
1 Jul 2025	2025 guidance withdrawn	—
Jul / Oct 2025	New 2025 forecast	~ $1.75 → ≥ $2.00 (actual $2.08)
6 Feb 2026	First 2026 guide	> $3.00
28 Apr / 28 Jul 2026	Raised twice	> $3.40 → > $4.80 (incl. ~$0.50 one-offs)
27 Oct 2026	Raised again (in force)	> $5.20

Reading: the December 2025 score sat between the collapse and the recovery: it bought a stock whose trailing numbers were wrong in both directions within twelve months.

Source: Centene 8-K earnings releases 2025-2026, SEC EDGAR

Factor view: what the position adds to the portfolio





Held at every rebalance since June 2021 — the value book's long-standing managed-care position.

Managed care was a large contributor to health-care selection (+1.3% a year for the sector in the backtest), but with episodes of sharp drawdown.

Policy risk is idiosyncratic and not captured by value, size or momentum factors: it should be sized as such.

Source: pipeline (pitches/assets/CNC_US; outputs/risk/brinson_gics_EW_total.csv)

Valuation and sensitivity

Price / 2026 earnings floor ($5.20) ≈ 13x; excluding ~$0.50 of one-offs ≈ 14x.

Scenario for 2027	Adjusted EPS	P/E	Value / share	vs $67.60
Bear	$3.50	11x	$38	-43%
Base	$5.00	13x	$65	-4%
Bull	$6.50	14x	$91	+35%

Bear: Marketplace pool worsens and Medicaid rates lag acuity. Base: 2026 run-rate minus one-offs. Bull: margin repair continues and buybacks resume.

Consensus: 21 analysts, 'Buy', average target $74.50 (third-party). The asymmetry is no longer attractive.

Source: own scenarios; StockAnalysis (29 Oct 2026, third-party); Centene Q3 2026 8-K

Catalysts and risks

Nov 2026 - Jan 2027: ACA open enrolment for 2027 (median premiums up ~15%).

Early Feb 2027: Q4 results and the first 2027 guidance — the real test of whether 2026 is a sustainable base.

2027: Medicaid work requirements; Medicare 2028 rate notice (Apr 2027); risk-adjustment data for 2026 (Jun-Jul 2027) — the same mechanism that triggered the 2025 withdrawal.

Risks: risk-pool deterioration, Medicaid rate adequacy, low Medicare star ratings, policy changes in Washington.

What would change our view: upgrade if 2027 guidance implies ≥ $5.50 of recurring EPS with Medicaid HBR below 93%; sell if risk adjustment or Medicaid acuity forces another withdrawal.

Source: Centene Q3 2026 8-K; CMS rate notices (Apr 2026); KFF via Fierce Healthcare (Oct 2026)

FAQ

"The stock went up 62%: wasn't the model right?" The outcome was right; the inputs were stale. A process judged on one outcome learns the wrong lesson.

"Why not shorten the accounting lag?" Announcement + 3 months was tested: results barely move. The deeper fix is to add estimate revisions or momentum, so that a collapsing earnings base is not mistaken for cheapness.

"Why keep it then?" Exiting would be a discretionary override of a rules-based book. The stock still screens as cheap and now fairly priced: we hold at the model weight and do not add.

"What did you learn?" For sectors where earnings can reset within a year (insurers, cyclicals), trailing multiples need a forward-looking or quality filter.

Source: outputs/backtest/robustness.csv (lag variant); own analysis

Place in the portfolio

2%	1.7%	+62%	+10.8%
equal-weight portfolio	minimum-variance portfolio	H1 2026 total return (EUR)	MSCI World Net EUR, H1 2026

USD exposure hedged under the IPS rule; no dividend, so no withholding drag.

Source: outputs/portfolio/weights_*.csv; pipeline returns

Sources

[1] Centene, 8-K Q3 2026 results and guidance, 27 Oct 2026 — sec.gov EDGAR (accessed 29 Oct 2026)

[2] Centene, 8-K FY2025 results and 2026 guidance, 6 Feb 2026; 10-K FY2025 (accessed 29 Oct 2026)

[3] Centene, 10-Q for the quarter ended 30 Sep 2026 (accessed 29 Oct 2026)

[4] Centene, 8-K guidance withdrawal, 1 Jul 2025 (accessed 29 Oct 2026)

[5]-[7] Centene, 8-K earnings releases Q3 2025, Q1 2026, Q2 2026 (accessed 29 Oct 2026)

[8] CMS, 2027 Medicare Advantage final rate notice, 6 Apr 2026

[9] Becker's, 2027 Star Ratings coverage, Oct 2026

[10] Reuters / Fierce Healthcare, Q3 2026 coverage, 27-28 Oct 2026

[11] Becker's, Fierce Healthcare and KFF on ACA subsidies and OBBBA, 2025-2026

[12] StockAnalysis, Centene price, multiples and consensus, 29 Oct 2026 (third-party)

Portfolio Project pipeline: z-scores, multiples (Bloomberg-derived), weights, returns

To provide / verify (pitches/sources/CNC_US/A_FOURNIR.md): initial 2025 guidance, FCF and net debt, OBBBA timing.